

UUID:64248A69-DA28-4CCD-ADDF-DB18EDA11EBF
TCGA-AR-A0U4-01A-PR

Redacted

1CD-0-3

Carcinoma, infiltrating duct, NOS 8500/3
Site: breast, NOS C50.9

Breast, right, simple mastectomy: Breast parenchymal tissue is negative for tumor. Focal calcifications identified. The nipple is negative for tumor. A single intraparenchymal lymph node is negative for tumor.

Breast, left, simple mastectomy: Infiltrating ductal carcinoma, Nottingham grade III (of III), [tubules 3 (of 3), nuclei 3 (of 3), mitoses 2 (of 3); Nottingham score 7-8 (of 9)] forming a 4.7 x 4.0 x 2.7 cm mass, located in the upper outer quadrant of the breast (AJCCpT2). Ductal carcinoma in situ is absent. Angiolymphatic invasion is absent. No calcifications are present. The tumor does not involve the nipple, overlying skin, or underlying chest wall. All surgical resection margins, including deep margin, are negative for tumor (minimum tumor free margin, 1.7 cm, deep margin).

Lymph nodes, left axillary sentinel, excision: Multiple (10) sentinel lymph nodes are negative for tumor. AJCC pN0(1-)(sn)

Immunohistochemical stains were performed on paraffin embedded tissue from the breast tumor using antibodies to ER, PR, and HER2/neu. Estrogen is negative, 0% nuclear staining. Progesterone is negative, 0% nuclear staining. HER2/neu protein overexpression is negative, score of 0, according to the interpretation guidelines in the FDA-approved HercepTest.

ER (Estrogen Receptor) test was developed and its performance characteristics determined by . It has not been cleared or approved by the U.S. Food and Drug Administration.

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Malignancy		
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	7/25	
Date Reviewed	9/1/10	